



Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health (MH) and General Health (GH) Divisions	Medical, Nursing, Allied Health, Facilities Management Staff, Operations Hub, Shift Engineers, Security, Patient Support Services, Work, Health & Safety

Policy statement

To outline the best practice response to all persons (e.g. patient, visitor, staff, or others) with a ligature in-situ, and provide guidance for clinicians in the safe use, storage, and replacement of ligature cutters.

Background

While risk can never be eliminated, all staff have a role in promoting a safe environment for all persons and implementing appropriate risk mitigation strategies to keep people safe. A person may attempt to self-ligate despite preventative strategies being in place. In these emergency situations, a ligature cutter may need to be used rapidly for the preservation of life or limb.

Staff should ensure their own safety before attending a ligatured person and conduct a risk assessment as per the procedures outlined in this policy.

Policy directives

- All services are responsible for minimising existing or potential ligature risks. Staff have a responsibility to understand ligature risks in their area or service, minimise these ligature risks or employ mitigation strategies
 - All staff working in areas where persons may be a risk to themselves must know how to activate an emergency response and the nature of the response in that clinical setting
 - Staff (including casual staff members) who are likely to encounter individuals at risk of ligature trauma complete relevant training or education
- Note:** Expectations will differ across services – e.g. mental health services, the Emergency Department (ED), State Major Trauma Unit (SMTU), Acute Medical Unit (AMU), Ward 10A, etc.
- All clinical staff who work in an area with access to ligature removal equipment must be aware of the equipment location and their roles and responsibilities as clinical staff.

Links within document

- [Appendix I: Ligature response flowchart - if ligatured person is accessible](#)

Principles

- Persons often exhibit signs of mental state deterioration prior to an event involving the application of a ligature
- All clinical staff are to be aware of and apply the principles of prevention, recognising and responding to acute deterioration in mental state and the Aishwarya's CARE Call process. Refer to the **Recognising and Responding to Acute Deterioration (RRAD) in Mental State, Cognition and Behaviour SOP** and **RRAD Policy**
- Identifying and responding to patients at risk of deliberate self-harm or suicide is in accordance with **Clinical Care of People Who May Be Suicidal Policy** and **WA Department of Health Principles and Best Practice for the Care of People Who May Be Suicidal**
- Any concerns about a person's imminent risk to themselves must be escalated for urgent clinical review. This may trigger the need for ligature removal equipment to be kept locally as part of a person's individualised, risk management plan
 - For GH services, consider liaising with treating team, Consultation-Liaison Psychiatry, Royal Perth Hospital (RPH) Safety All Hours for Everyone (SAFE) or Bentley Health Service (BHS) ^{Be}SAFE for input and to determine if ligature equipment is to be deployed to the relevant ward
- In all cases, clinical handover must include information exchange between all disciplines using the iSoBAR format.

Definitions

Ligature	Any material used to tie or bind tightly to any parts of the body, e.g. the neck.
Ligature Point	Any fixture or fitting or item which is load bearing (wholly or partially) that can be used to tie or secure a cord, belt, sheet or other tether for the purpose of ligature use, i.e. as a means of self-harm, self-strangulation and in extreme circumstances result in death by suicide.
Strangulation	Strangulation occurs when external pressure on the neck obstructs blood vessels and air passages . Vessels become occluded at much lower external pressures than the airway, so the majority of strangulations are due to compromise of blood flow to the head rather than actual airway compression.
Ligature cutter	Any device specifically used to cut through a ligature while minimising harm to the part of the body that a ligature is attached to, i.e. rescue hook, trauma scissors, wire or bolt cutters. The devices must not be used for any other purpose than dealing with a ligature in an emergency situation ^{1,2} The effectiveness of ligature cutters are largely dependent on the sharpness of the blade, and thus, require sharpening after use (in accordance with manufacturers instruction) or may be single use items (i.e. disposed of after use). ²
A Rescue Hook (or similar)	A tool which is specifically used to cut ligatures such as webbing, rope, nylon line, leather, electrical flex, and clothing. ^{1,2} 
Wire/Bolt Cutter	A tool used to cut a metal cord, e.g. antenna cable from TV. ³ 
Trauma Scissors (e.g. Tuff Cut)	Heavy duty shears used for clothing, seat belts, plastic ties, ropes, leather, and denim. ² Trauma scissors can be used as a backup to wire or bolt cutters. 

Procedure details

Responding to a person with a ligature in-situ

On discovering a person with a ligature in situ staff (First Responder) must conduct an immediate risk assessment, including the following:

Person:

- Type of ligature and location on body
- Conduct immediate assessment of person's signs of life/physiological state
- Behaviours the person is displaying that may be an imminent risk to staff/others' safety e.g. aggressive, confused, unpredictable
- Are they heavy and/or wet and slippery?

Environment:

- Location of person – e.g. are they accessible? Is key, lock release disc for applicable areas or door override required?
- Hazards such as live electrical cables being used as a ligature, water, other persons that may be distressed or at risk.

Task:

- Whether the person is partially or fully suspended
- Level of response required: person may have ligature around neck and be fully suspended, with imminent life threat or have attached a ligature around other part of the body that is restricting blood flow with no immediate life threat but potential for threat to limb
- If person is partially or fully suspended, evaluate resources or equipment needed to aid safe handling process during response.

Staff:

- Assess level of response required – emergency response vs local response
- Assess number of staff required to safely complete task
- Assess need for other internal services – Facilities or Shift Engineer
- Assess requirements for external support – Fire Service/Police. If a person is resistive to intervention, contact Security to provide restrictive practice or crowd control (activate Code Black), if required. Security staff are not expected to remove a ligature and are to liaise and work alongside clinical staff and WA Police as required.

Procedure details – Responding to a person with a ligature in-situ cont'd

For fully or partially suspended strangulation

• **Call for emergency assistance**

- RPH or BHS call '55' and activate Code Black and/or Blue
- City East & Midland Community Mental Health Services (CMHS) call **000**
- For persons discovered outside of hospital or health service boundaries call **000**
- For persons not able to be safely accessed by staff, Emergency Services (000) may also need to be contacted in conjunction to local emergency responses.

For ligatured body parts or non-suspended ligature

Emergency assistance may not be required – assess person and risk to life or limb and activate local assistance or Code Blue or Code Black as required.

For home visiting, refer to processes for responding to persons experiencing a critical incident in the community or during a home visit, e.g. refer to EMHS Community/Home Visiting Policy, RPBG Community Mental Health Home Visiting SOP, Leave for Mental Health Patients Policy, and Leave for General Health Patients Policy.

- Remain with the person with a ligature in situ
- Delegate a responder to collect the ligature removal equipment. In the absence of assistance, the First Responder may be required to collect the equipment themselves
- If trauma scissors are available, initial response is to attempt to cut the ligature between the ligature point and the person with trauma scissors, if suitable, before the ligature removal equipment is obtained or arrives. For GH areas, trauma scissors are co-located with the Resuscitation Trolley
- **For areas with no ligature removal equipment – for inpatients, SAFE or BeSAFE teams will be aware of all locations of ligature removal equipment across the site and delegate a staff member to collect from the nearest location.**

When safe to do so:

- Approach the person – if person is resistive or aggressive or resists attempts to remove the ligature, restrictive practice such as restraint may be required to help ensure the safe removal of the ligature. These interventions should be sensitive to the needs of the person. If the person is trying to remove the ligature, advise them to cease this action and assure them that staff will use appropriate interventions to assist them
- Provide immediate clinical support, if possible, manually remove the ligature or release tension from the ligature. See [Procedures for suspended strangulation](#) section
- If the person is suspended, use safe handling principles to secure the person and relieve tension from the ligature– this may need to involve more than one person or a mechanical safe handling device

Procedure details – Responding to a person with a ligature in-situ cont'd

- It may be necessary to release the person from the ligature without support to reduce the time taken until medical care can be given
- Use the appropriate ligature removal device in accordance with training and manufacturer's instructions
- If person is partially or fully suspended the ligature should be cut between the ligature point and person
- Perform any required immediate medical intervention as per **Medical Emergency and Cardiorespiratory Arrest Management Clinical Practice Standard (CPS)**
 - All episodes of strangulation should be treated as a suspected spinal injury – immobilise neck as per **Patient with an Acute Spinal Injury CPS**
 - Provide immediate psychological support to the person.

If unable to assure the safety of all responders or remove the ligature it may be necessary to await further assistance or support i.e. arrival of Police, Fire Service.

Procedure for suspended strangulation

Risk Assessment: Staff should NOT lift or hold a suspended person's weight or attempt any technique or manoeuvres they feel would be hazardous to them. Staff should carry out a dynamic risk assessment and apply safe handling principles to the best of their ability in the situations they find themselves. Efforts should be made to stabilise and guide the person's head and body to the floor if possible. All suspended patients should have c-spine precautions once lowered.

Points to consider:

- Priority is to release pressure on the neck and remove the ligature as soon as it has been assessed as safe to do so
- Consider/assess the weight and height of the person suspended
- Height of suspension and if the person is fully or partially suspended
- Accessibility of person and ligature
- Surface person is being lowered/landing onto.

Partially suspended strangulation (sitting or kneeling)

If possible, elevate the head and torso to relieve pressure on the ligature. **Do not** pull on the ligature to remove or unhook it. When head is supported, or risk assessment determines an unhindered drop is warranted – cut ligature at the central point between ligature point and neck. If supported, lower person to ground. Remove neck ligature as per [Using ligature cutters](#) section.

Partially suspended strangulation (lying)

Slide the person towards the point of suspension, using slide sheets, if possible, to reduce the tension on the ligature before removal.

Procedure for suspended strangulation cont'd

Complete suspended strangulation

Where accessible (e.g. this may not be possible in a stairwell), position any nearby suitable equipment/furniture, (e.g. stretcher or bed or fixed chair without wheels) under suspended person to support them. The head and neck should be stabilised if possible. As soon as weight is supported, or staff are clear if unhindered drop is warranted - cut ligature at the central point between ligature point and neck.

If ligature point is inaccessible i.e. due to height of suspended person Emergency Service response is appropriate (000).

Mechanical aids such as a ladder may be required (ladders are available from RPH Shift Engineer on call, BHS Shift Engineer Monday- Friday and Security after hours (A/H). CMHS or Outpatients /non-hospital sites will require 000 assistance from Police or Fire Service to access ligature point. Other areas may require 000 assistance dependant on accessibility and height of ligature.

Using ligature cutters

Ligature cutters or scissors to be used in accordance with the manufacturer's instructions and training.

- **Do not** pull on the ligature to remove or unhook it, e.g. – from over a tap or door handle
- Assess material used for ligature and select appropriate cutter i.e. if electrical cable used for ligature wire or bolt cutter may be required
- If fully or partially suspended strangulation, the ligature should first be cut between the ligature point and the person before attempting to release it from the neck
- When a ligature is located around a person's neck, make the cut at the side of the neck (if possible) as the natural soft tissue and hollows of the neck may facilitate easier insertion of the cutter. Cut ligature quickly at the **thinnest** point distant from any knots or away from the trachea
- Avoid cutting the knot, as this is harder to cut and may be required for forensic investigation
- When using the **rescue hook**, place the rounded or blunt end between the skin and the material to be cut at a slight angle. Cut with a sawing motion using the sharp cutting edge on the inside of the blade.



Using ligature cutters cont'd

- When using the **trauma scissors or wire or bolt cutters** apply significant downward pressure. (Keep fingers away from contact with bladed edge)
- If necessary, a combination of cutting devices (i.e. wire cutters and/or trauma scissors) can be used. If ligature cutter is deemed to be ineffective seek backup equipment from alternative location.

Post ligature removal

Following an incident where a ligature has required removal.

Clinicians are to provide or ensure an immediate medical review including:

- Physiological assessment
- Mental state review
- Review of current treatment setting:
 - Transfer to inpatient setting
 - Need for transfer to ED or other inpatient area for further medical management
 - Transfer to more secure or safe inpatient environment
- Review of level of visual observations
- Revise *Risk Assessment and Management Plan (RAMP)* and/or *Progressive Risk Assessment (PRA)* (MH services) or *Behavioural Observation Form (BOF)* [GH services]) and management plan as relevant.

Person critically injured or deceased

In the event the person is critically injured or deceased the following processes must be conducted.

- If the person is subsequently pronounced dead, use privacy screens if in a public space. The Police will treat this death as unexplained, and area is treated as a crime scene until proven otherwise (see **EMHS Reporting and Care of the Deceased Policy**)
- Preserving forensic evidence
 - Retain ligature cutters in suitable sharps container for Police
 - Do not touch or disturb the area or other evidence until the police give permission. If this now means emergency equipment is unavailable, staff must ensure they have access to replacement emergency equipment as soon as practical, the nurse in charge must communicate this with the surrounding wards and relevant Senior RN (e.g. Nurse Unit Manager [NUM] or SAFE or ^{Be}SAFE)
- Retain ligature cutters or all ligature parts (preserve any ligature knot) or if part of ligature remains attached to a ligature point, do not remove until this has been authorised by the Police or appropriate senior staff (i.e. NUM or Shift Coordinator or Team Leader)
- Preserve the room or space, do not remove any items
- Lock or secure area until further instructions from senior managers or Police or Coroner

Person critically injured or deceased cont'd

- In the event of death, complete requirements in accord with the **EMHS Reporting and Care of the Deceased Policy** and the [Chief Psychiatrists' notifiable incident report \(MH persons\)](#), as applicable. Comply with external requirements and support for any investigation in the case of death of a person
- Medical Officer (MO) or Duty Medical Officer (DMO) is to liaise with the WA Police Coronial Investigation Squad in regard to the process for contacting the person's next of kin (NOK) or personal support person (PSP) or guardian or nominated person or other as outlined in the **Mental Health Act (MHA) 2014**.

Staff:

Inform NUM or Clinical Nurse Specialist (CNS) or Shift Coordinator or Team Leader or Other Outpatient or CMHS staff

Senior clinicians or management will ensure immediate support and appropriate debriefing for the staff. Staff are to encourage the person or other witnesses to receive debriefing or psychological support post incident due to the risk of adverse psychological trauma.¹ Refer to

- **EMHS Post Incident Management / Critical Incident Stress Debriefing Policy or Employee Assisted Program (EAP) and RPBG Mental Health Consumer, Carer and Visitor Support Post-Critical Incident SOP**
- Complete clinical handover for all shifts or work areas
- Complete:
 - Datix Clinical Incident Management System (CIMS) report and;
 - Office of the Chief Psychiatrist Notifiable Incident report where required, and/or
 - Combined Hazard or Incident Report (CHoIR) where applicable
- Document the incident in the patient's integrated notes and add alert in iCM or Psychiatric Services On Line Information System (PSOLIS)/WebPSOLIS (MH persons)
- Consider need to report to Work Health and Safety (WHS) (who may need to advise WorkSafe in accordance with **WHS Act 2020**)
- **RPBG does not have facility to sharpen cutters. Treat all devices used for ligature removal as single patient use**
- For ordering ligature removal equipment items refer to the [Ligature removal equipment checks section](#).

Others:

- Any other persons or witnesses are to be offered or provided immediate emotional support and debriefing.

Location and storage of ligature cutters

Ligature cutters must be easily accessible to staff but inaccessible to other persons, i.e. patients or visitors (must be stored in locked area). Ligature kits will differ across the following areas:

MH

- All MH inpatient areas must have available standardised ligature removal equipment (single use) consisting of bladed ligature cutters (rescue hook), wire or bolt cutters, trauma scissors and be kept together (trauma scissors are also available as part of standard resus trolleys on GH wards).
- **Authorised MH inpatient areas** – 1 ligature removal kit in **each ward and therapy area** (stored in nursing station (consider wall mounted and/or enclosed in clear, secure bags¹) or trauma scissors stored in standard resus trolleys
 - Ward 10 (BHS only) – Stored in Treatment room
- **Unauthorised MH inpatient areas** – 1 in each ward (stored in resus trolleys in nursing station)
Exception:
 - MH Bidi Wungen Kaat Centre (Transitional Care Unit [TCU]) stored in designated, office area and treatment room
- **CMHS - 2 in each area** (as access to replacements may be delayed due to isolated location) rescue hook and trauma scissors only in standard resus trolleys and stored:
 - Teams based at City East CMHS or Midland CMHS - Wellness Clinic Area (Nursing station) or First aid kits
 - Teams based at BHS – stored in Treatment room
 - Additional trauma scissors are stored in the Automated External Defibrillator carry case.

GH

- **Identified high risk GH areas as nominated by Coordinators of Nursing (CoNs) or Co-Directors** (e.g. ED, AMU, SMTU, and Ward 10A) must have available standardised ligature removal equipment (single use) consisting of bladed ligature cutters (rescue hook), wire or bolt cutters, trauma scissors and be kept together
 - **1 x ligature kit in each area** (stored in locked Treatment rooms) or trauma scissors stored in standard resus trolleys
- **GH low risk inpatient areas as nominated by CoNs and Co-Directors** – 1 in each area consisting of rescue hook and trauma scissors only in standard resus trolleys
- **GH Outpatients** to access via **SAFE or BeSAFE** (refer to **Medical Emergency and Cardiorespiratory Arrest Management (MET) CPS**).

Replacement of used Rescue Hooks can be obtained from Product Liaison Officer (central store), SAFE / BeSAFE, or Central MH area.

Ligature removal equipment checks

NUMs or CNS or Team Leaders or Manager Community or Outpatient Services are to:

- Ensure ligature checks are completed as part of the weekly resuscitation checklist. (Refer to **Medical Emergency and Cardiorespiratory Arrest Management CPS**– Checking of the Resuscitation Trolley) and immediately after every use by Shift Coordinator (inpatient) or Care Coordinator or Wellness Clinic staff (CMHS). For areas that do not have a resuscitation trolley, staff will complete checks as part of the monthly first aid checklists and after use
 - Check that each item is present and ready for immediate use ie. removed from any original boxed packaging
 - Ensure rescue hook is secured in black nylon pouch with velcro closure – do not open/remove to avoid contamination or accidental injury
 - **Note:** Trauma scissors and rescue hooks are single patient use - central store is available for restocking of used rescue hooks only. **NOTE:** trauma scissors (located on the resus trolley) may be used for opening packaging and in this case can be re-used.
- Escalate any concerns relating to the equipment to Shift Coordinator or Team Leaders or Program Managers for all MH and GH Outpatients services, as required
 - Should any part of the tool become damaged or missing immediately arrange a replacement via the NUM or Team Leader or Shift Coordinator or SAFE/^{Be}SAFE
- Order ligature removal equipment as per process for non imprest items.

Ligature removal equipment must not be removed from designated locations without ensuring all staff in the department or service are aware, except for use.

Training

- Training relating to the risk management and safe response for persons with a ligature in-situ; and storage of ligature removal equipment must be provided to the following groups:
 - Within **MH inpatient areas and CMHS**
 - Staff working in identified **high risk GH inpatient services, i.e. ED, AMU, SMTU, and Ward 10A**. High risk general health areas will be identified through environmental and patient risk conducted by NUM or Team Leader and confirmed by Co-Directors or CoNs or Line Managers
 - SAFE / BeSAFE / Emergency Response Team
- Training and orientation to the location of ligature removal equipment will be provided as part of ward /role orientation and signed off as part of the ward learning map with an annual refresher provided either as part of annual mandatory training (for MH), or local ward training (identified high risk GH areas).
 - Security staff are to receive orientation by Line Manager to gain awareness of the location of ligature removal equipment across sites, this policy and to assist with environmental safety or crowd control
- Agency or casual staff will be orientated to the location of ligature removal equipment, process for responding to a person with a ligature in situ and local emergency response procedures as part of unit/ward orientation
- Take 5 Ligature educational resource available for all staff on [RPBG Take 5 HealthPoint page](#).

Workplace inspection & audit requirements

Complete:

- For MH services and designated GH wards and/or CMHS or Outpatient clinics, an annual ligature audit to identify
- Equipment readiness
- Equipment checks completed
- Ligature or ligature point concerns and resolving or mitigating ligature-related concerns
- Annual training KPI.

RDBG MH staff to use versions of the Statewide Standardised Clinical Documentation (SSCD) that are on PSOLIS or when they become available on PSOLIS. For documents that are not available on PSOLIS, staff must complete the SSCD documents in paper-based or sanctioned writable PDF format. Refer to the **WA Department of Health 'Triage to Discharge' Mental Health Framework for SSCD Guidelines** for details of which SSCD documents are available on PSOLIS.

Compliance monitoring

- The appropriate Service Lead (NUM or Team Leader or CNS or SAFE or ^{Be}SAFE) is responsible for ensuring:
 - Compliance with training KPIs (annual review)
 - All clinical staff are aware of the availability and location of the ligature removal equipment (monitored via ward orientation checklist/processes)
 - Compliance with weekly (minimum) equipment checks.

Compliance will be monitored by NUM or Team Leader or CNS or SAFE or ^{Be}SAFE via

- Datix CIMS system process and/or CHoIR (e.g. clinical incidents related to ligature incidents or assessments, suicide attempts or suicides or deaths) and/or Chief Psychiatrists' notifiable reports, if appropriate
- Annual audit of Resuscitation Trolley conducted by MET Service (CNC RPH or CNS BHS) will capture ligature removal equipment checks.

Facilitator

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy through consultation with the Aboriginal Health Strategy Team and the Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1559).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Clinical Handover Policy](#)
- [Behaviours of Concern \(Preventing, Recognising and Responding\) Practice Standard](#)
- [Prevention and Management of Aggressive Behaviours in Mental Health Inpatient Settings SOP](#)
- [Patient Property Management Policy](#)
- [Community Mental Health Home Visiting SOP](#)
- [Allied Health Home Visiting SOP](#)
- [Escalation to Consultant Policy](#)
- [Special and Supportive Observations Policy](#)
- [Mental Health Consumer, Carer and Visitor Support Post-Critical Incident SOP](#)
- [Medical Emergency and Cardiorespiratory Arrest Management \(MET\) CPS](#)
- [Telehealth SOP](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Patient with an Acute Spinal Injury CPS](#)
- [RRAD in Mental State, Cognition and Behaviour SOP](#)
- [RRAD Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Leave for Mental Health Patients Policy](#)
- [Leave for General Health Patients Policy](#)

EMHS Policy Hub

- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [Reporting and Care of the Deceased Policy](#)
- [Post Incident Management / Critical Incident Stress Debriefing Policy](#)
- [Acute Response Mental Health Emergencies / Crisis Policy](#)
- [Community / home visiting Policy](#)

WA Department of Health Policy Hub

- [Clinical Incident Management Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- ['Triage to Discharge' Mental Health Framework for SSCD Guidelines](#)

Related policies, practice standards, clinical guidelines cont'd

Other

- [Chief Psychiatrist's Authorised Hospitals Standards - Chief Psychiatrist of WA](#)
- [Mental Health Act 2014](#)
- [Work Health and Safety Act 2020](#)
- [Chief Psychiatrists Standards of Clinical Care](#)
- [RPH Emergency Procedures on HealthPoint](#)
- [BHS Emergency Procedures on HealthPoint](#)

Links to resources

- [Take 5: Ligature Risk, Awareness and Response](#)
- [MyLearning: Ligature Risk, Awareness, and Response](#)

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy Hub.

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021): Standard 1: Clinical Governance; Standard 5: Comprehensive Care; Standard 6: Communicating for Safety; Standard 8: Recognising and Responding to Acute Deterioration

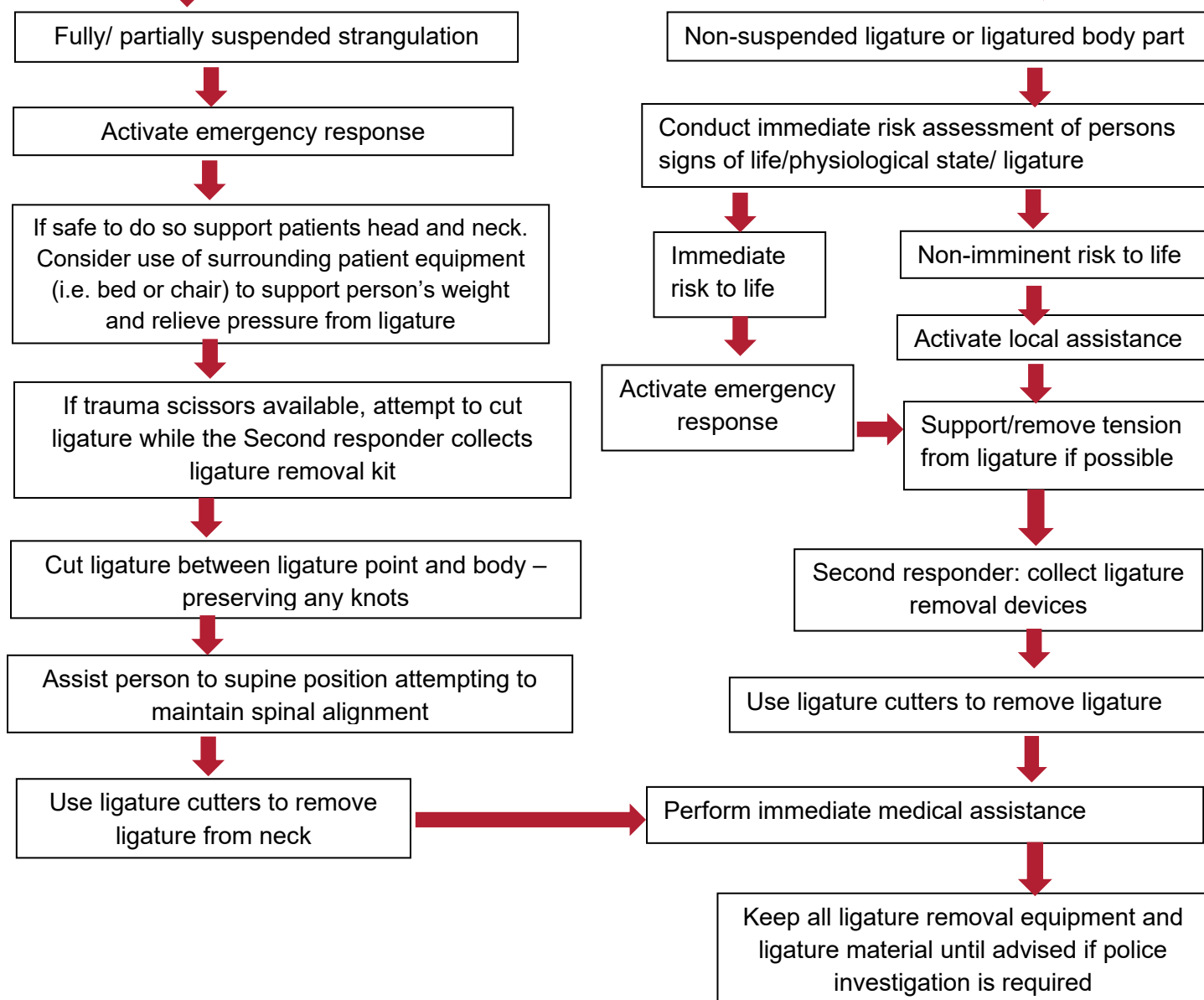
NSMHS Standards (2010): Standard 2: Safety; Standard 8: Governance, leadership and management; Standard 10: Delivery of care

References

1. NHS Foundation Trust. (2019). *Using Ligature Cutters in Mental Health and Learning Disability Services Standard Operating Procedure*. London, England.
2. NHS Foundation Trust. (2017). *Safe Use of Ligature Cutters Standard Operating Procedure*. South Staffordshire and Shropshire Healthcare. London, England. [Safe Use and Storage of Ligature Cutters - Standard Operating Procedure \(SOP\) - PDF Free Download \(docplayer.net\)](#)
3. Government of Western Australia. (2019). *Safe Use and Storage of Ligature Cutters procedure*. North Metropolitan Health Service Mental Health Area. Perth, Western Australia.
4. Government of Western Australia. (2020). *Environmental Safety Audit Procedure*. North Metropolitan Health Service Mental Health, Public Health and Dental Services. Perth, Western Australia.
5. Queensland Government. (2012). *Guideline for Managing Ligature Risks in Public Mental Health Services*. Queensland Health. Queensland.

Appendix I: Ligature response flowchart – if ligatured person is accessible

On discovering a person with a ligature in situ



Remove used ligature cutter from circulation and replace with new ligature cutter – access as per [Location and storage of ligature cutters](#) section as soon as is practicable.

In situations where the person resists the staff actions to remove the ligature, it might be appropriate to restrict the person's ability to struggle, especially where the struggling behaviour increases the risk(s) presented by the ligature, or by the use of the ligature cutter by staff. A Code Black may be required.